Fax

TO	**MEDICAL SUPPLIES SINGH**	SPECIALTY	x Unknown or N/A
PHONE	(248)924-9406	FAX	(734)864-9041
FROM	**Arqam Zia MD at Canton Endocrinology**	SPECIALTY	Endocrinology
PHONE	7349998067	FAX	7343987551
SUBJECT	**Patient Referral**	TIME	01:04:30 AM
DATE	12/30/2025		

Arqam Zia MD sent you this referral using Practice Fusion's EHR. To retrieve this referral instantly online, sign up for a Practice Fusion account today.

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PATIENT
BERNARD LEESE
DOB 09/19/1945
AGE 80 yrs
SEX Male
PRN LB1832

H N/A
M (734) 391-5541
W N/A
E Tommysmumma@yahoo.com
43345 TYLER RD
APT #15
BELLEVILLE, MI 48111

CANTON ENDOCRINOLOGY
T 7349998067
F 7343987551
1600 S CANTON CENTER RD
SUITE 2200
CANTON, MI 48188

Referrals/Response Letter

To: MEDICAL SUPPLIES SINGH
From: Arqam Zia MD
Sent: 12/30/2025 01:04:30
Subject: Patient Referral
Regarding: Bernard Leese

freestyle libre 3 plus
(patient on insulin)

Sincerely,

Arqam Zia MD

Insurance

PRIMARY PAYER

NAME	Medicare Plus Blue	**ADDRESS**
PLAN	Other - Medicare Plus Blue	
GROUP ID	007032423	
INSURANCE ID	X3LM68720773	

Diagnoses

TYPE	CODE	DESCRIPTION	START/STOP
ICD-10	E10.65	Type 1 diabetes mellitus with hyperglycemia	12/02/2025 -
ICD-10	Z96.41	Presence of insulin pump (external) (internal)	12/02/2025 -
ICD-10	E55.9	Vitamin D deficiency, unspecified	12/02/2025 -

Active Medications for Bernard Leese

MEDICATION	SIG	START/STOP	ASSOCIATED DX
Fludrocortisone Acetate 0.1 MG Oral Tablet - Fludrocortisone Acetate Oral Tablet 0.1 MG	Take 3 tablets by mouth daily	N/A -	
Gabapentin 300 MG Oral Capsule - Gabapentin Oral Capsule 300 MG	Take 3 capsules by mouth daily	N/A -	
Aspirin 81 MG Oral Tablet Chewable - Aspirin Oral Tablet Chewable 81 MG	Chew and swallow 1 tablet by mouth daily	N/A -	
Colace 100 MG Oral Capsule - Docusate Sodium Oral Capsule 100 MG	Take 1 capsule by mouth 2 times per day	N/A -	
HYDROcodone-Acetaminophen 5-325 MG Oral Tablet - Hydrocodone-Acetaminophen Oral Tablet 5-325 MG		N/A -	
Magnesium Oxide -Mg Supplement 400 (240 Mg) MG Oral Tablet - Magnesium Oxide (Mg Supplement) Oral Tablet 400 (240 Mg) MG	Take 1 tablet by mouth 1 time per day with food	N/A -	

Midodrine HCl 10 MG Oral Tablet - Midodrine HCl Oral Tablet 10 MG		N/A -
Klor-Con M20 20 MEQ Oral Tablet Extended Release - Potassium Chloride Microencapsulated Crystals ER Oral Tablet Extended Release 20 MEQ	Take 1 tablet by mouth daily with food	N/A -
Crestor 20 MG Oral Tablet - Rosuvastatin Calcium Oral Tablet 20 MG		N/A -
Farxiga 10 MG Oral Tablet - Dapagliflozin Propanediol Oral Tablet 10 MG	Take 1 tablet by mouth daily	N/A -
Dexcom G7 Sensor - Continuous Glucose System Sensor	CHANGE SENSOR EVERY 10 DAYS , TYPE 1 DM , E10.65	N/A -
Lantus SoloStar 100 UNIT/ML Subcutaneous Solution Pen-injector - Insulin Glargine Subcutaneous Solution Pen-injector 100 UNIT/ML	15 units once a day	N/A -
HumaLOG KwikPen 100 UNIT/ML Subcutaneous Solution Pen-injector - Insulin Lispro Subcutaneous Solution Pen-injector 100 UNIT/ML	2 to 7 units before meals three time per day	N/A -

There is no allergy history recorded for this patient

Encounter - 12/02/2025

SEEN BY		SEEN ON	
Arqam Zia MD		12/02/2025	
HEIGHT	WEIGHT	BMI	BLOOD PRESSURE
70.0 in	139.0 lbs	19.9	78/49
TEMP	PULSE	RESP RATE	HEAD CIRC
N/A	81.0 bpm	16.0 rpm	N/A

CC

(Appt time: 12/2/2025 11:00:00 AM) (Arrival time: 10:56 AM)

S

patient is here with her daughter / Jill Guisgand
PCP Priti Bhardwaj (Lincoln Park Michigan)
patient now lives Assisted living facility HAMPTON MANOR at Van Buren Township since 10/1/2024
(previous endocrinologist Dr Gaurav Bhalla 810-216-4200)
- Reports some episodes of hypoglycemia, particularly at night, with readings as low as 58 mg/dL. However, these may be false lows (compression lows) due to pressure on the sensor while sleeping. Bedtime snack consists of a half sandwich with Miracle Whip, bread, and butter pickles.

- Meals are provided and are reportedly improved, consisting of a double protein portion with low-carbohydrate vegetables. A doctor's order is required for a lower carbohydrate diet as the facility was previously providing high-carbohydrate meals.
- Current medications: Lantus 15 units daily at 9:00 AM. Sliding scale insulin regimen: before meals and at . Bedtime , fast-acting insulin is also administered by the facility.
- Social history: Lives with his wife (age 88) in a one-bedroom apartment at the assisted living facility. Meals, cleaning, and medication administration are provided by the facility staff. Family is involved in his care.
CORRECTION / SLIDING SCALE FOR Humalog OR Novolog (short acting insulin) before each meal three times per day and at bedtime
inject insulin per scale

101-140 : 2 units

If 141-170 : 3 units

If 171-200 : 4 units

If 201-250 : 5 units

If 251-300 : 6 units

If 301-400 : 7 units

If 401-450 : call md
LANTUS 15 UNITS AT 9 AM

PMH
diagnosed with cancer of upper and lower jaw , right side , s/p extensive surgery , s/p radiation , lost hearing after radiation (surgery 6/2023)

he has DM since age 40 yrs
he was told he is T1DM as there is no insulin production
he was in the hospital 2/2025 , he broke left hip , he was discharged on 2/12/2025
h/o recurrent falls
h/o low potassium
PMH

anemia
Sq cell cancer
orthostatic hypotension
PVD
CAD
HL
CABG in 2018
thrombocytopenia
vitamin D def
Meds

REVIEWED / SCANNED
=====
REVIEW OF SYSTEMS today

HEENT: Denies vision problems. . No dysphagia, odynophagia. No lumps in neck. No epistaxis. No hemoptysis..

Cardiac: Denies CP, CPOE, palpitations.

Respiratory: Denies SOB, SOBOE, cough, hemoptysis, wheeze.

GI: No NVD, no diarrhea, no melena, no hematochezia, no constipation. BMs regular. Denies abdominal pain.

GU: Denies dysuria, hematuria, incontinence.
MSK: Denies swelling or pain.

Neuro: Denies seizures, LOC, ataxia or vertigo symptoms.

Skin: Denies rashes, jaundice or lesions. .

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LABS:
11/21/25
Sodium Level: 139
Potassium Level: 4.2
Chloride: 104
Bicarbonate Level: 25
Anion Gap: 10
Blood Urea Nitrogen: 19
Creatinine: 1.08
eGFR By Creatinine: 69
Glucose Level: 192 (H)
Calcium Level Total: 8.5
Alkaline Phosphatase: 97
Albumin Level: 3.0 (L)
A/G Ratio: 0.9
Protein Total: 6.4
Globulin: 3.4

Aspartate Aminotransferase (AST): 21
Alanine Aminotransferase (ALT): 10
Bilirubin Total: 0.3
==========

07/24/25
Sodium: 138
Potassium: 4.1
Chloride: 101
CO2: 30
Anion Gap: 7
Glucose: 201 (H)
BUN: 21
Creatinine: 1.05
Calculated GFR: 72
BUN/Creatinine Ratio: 20.0
Calcium: 8.6
AST (SGOT): 23
ALT (SGPT): 11
Alkaline Phosphatase: 105
Total Protein: 7.2
Albumin: 3.6
Total Bilirubin: 0.3
Cholesterol: 111
HDL: 44
Triglycerides: 82
VLDL Cholesterol Cal: 16.4
LDL Calculated: 51
Non HDL Chol. (LDL+VLDL): 67
Chol/HDL Ratio: 2.5
Vit D, 25-Hydroxy: 25.9 (L)
Hemoglobin A1C: 7.5 (H)
Mean Bld Glu Estim.: 169

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02/06/25
eGFR By Creatinine: 90
Sodium Level: 137
Potassium Level: 3.6
Chloride: 102
Bicarbonate Level: 24
Anion Gap: 11
Blood Urea Nitrogen: 19
Creatinine: 0.79
Glucose Level: 74
Calcium Level Total: 8.2 (L)
Magnesium: 1.7

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02/04/25
eGFR By Creatinine: 72
Sodium Level: 137
Potassium Level: 4.5
Chloride: 101
Bicarbonate Level: 27
Anion Gap: 9
Blood Urea Nitrogen: 20
Creatinine: 1.05
Glucose Level: 123 (H)
Calcium Level Total: 8.3 (L)
Magnesium: 1.8
Alkaline Phosphatase: 94
Albumin Level: 2.8 (L)

A/G Ratio: 0.7
Protein Total: 6.7
Globulin: 3.9 (H)
Aspartate Aminotransferase (AST): 27
Alanine Aminotransferase: 12
Bilirubin Total: 0.4

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O

Vitals: reviewed vital signs above.

General: in no acute distress,

Mood : normal affect and speech.

Skin: no rash

Eyes: EOM's full, conjunctivae clear.

Neck: Supple,

Thyroid : No goiter

Heart: no tachycardia

Chest: no apparent respiratory distress

Abdomen: unremarkable

Extremities: no edema

A

see below

Diagnoses attached to this encounter:

Type 1 diabetes mellitus with hyperglycemia [ICD-10: E10.65], [ICD-9: 250.81], [SNOMED: 367991000119101]
Presence of insulin pump (external) (internal) [ICD-10: Z96.41], [ICD-9: V45.85], [SNOMED: 450657002]
Vitamin D deficiency, unspecified [ICD-10: E55.9], [ICD-9: 268.9], [SNOMED: 34713006]

P

1. TYPE 1 DM
Free style libre 3 plus sensor DATA AS BELOW

November 19, 2025 - December 2, 2025 14 Days
Time CGM Active: 58%
Ranges And Targets For Type 1 or Type 2 Diabetes
Glucose Ranges Targets % of Readings (Time/Day)
Target Range 70-180 mg/dL Greater than 70% (16h 48min)
Below 70 mg/dL Less than 4% (58min)
Below 54 mg/dL Less than 1% (14min)
Above 180 mg/dL Less than 25% (6h)
Above 250 mg/dL Less than 5% (1h 12min)
Each 5% increase in time in range (70-180 mg/dL) is clinically beneficial.
Average Glucose 160 mg/dL
Glucose Management Indicator (GMI) 7.1%
Glucose Variability 32.6%

1. Type 2 Diabetes Mellitus
- Assessment: Glucose control is variable with periods of hyperglycemia and potential false hypoglycemia. Current regimen appears to be working reasonably well.
- Investigations planned: Will provide lab orders for A1C and TSH to be completed before the next visit.
- Medical treatment planned:
- Continue Lantus 15 units daily at 9:00 AM.

- Continue sliding scale insulin regimen.

CORRECTION / SLIDING SCALE FOR Humalog OR Novolog (short acting insulin) before each meal three times per day and at bedtime
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If 401-450 : call md
- Prescriptions for Lantus and Humalog pens sent to Walgreens in Belleville.
- Continue bedtime snack to prevent nocturnal hypoglycemia.
- Lifestyle modifications:
- Continue current diet with approximately 45-65 grams of carbohydrates per meal.
- Follow-up appointments:
- Follow-up in 3 months with blood work prior to the visit. Will call if any changes occur sooner.

2. Insulin Pump Management
- Assessment: Currently not using his Medtronic pump (model 780G). Family is in the process of getting it reactivated via Carelink. Training will be required for Bernard and his family on pump use, including carb counting for meals (breakfast, lunch, dinner).
- Plan:
- Proceed with reactivating the Medtronic pump.
- Once activated, will attempt to resume pump therapy.
- If pump therapy is not successful or tolerated, will revert to current multiple daily injection regimen.
- Patient education on the diagnosed condition:
- Educated on compression lows (false hypoglycemia) caused by pressure on the sensor, especially when sleeping on his side.

- Explained that insulin takes 15-20 minutes to be absorbed and remains active for 3-4 hours, which is why it is taken before meals.
- Instructions for monitoring and managing symptoms:
- Continue bedtime snack to mitigate nocturnal hypoglycemia.
- If symptomatic hypoglycemia occurs, treat with orange juice and a banana.

(level of visit) : A comprehensive history; A comprehensive examination; Medical decision making of high complexity. the presenting problem are of moderate to high severity. 60 minutes spent ..

Tests ORDEREDHemoglobin A1c
Comprehensive metabolic 2000 panel - Serum or Plasma
Vitamin D 25-Hydroxy
Microalbumin/Creatinine panel in random Urine
Lipid panel with direct LDL - Serum or Plasma
TSH

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(level of visit) : A comprehensive history; A comprehensive examination; Medical decision making of high complexity. the presenting problem are of moderate to high severity. 60 minutes spent ..
Bernard Leese is advised to make follow up visit in 12 weeks , however to call sooner if any questions or concerns or email me thru patient portal
Thank you for referring Bernard Leese to me for evaluation and treatment.

If any questions please free to contact us.

Arqam Zia MD
Diplomate American Board of Endocrinology,Diabetes and Metabolism
CANTON ENDOCRINOLOGY
Diabetes Thyroid & Endocrinology Clinic

1600 S Canton Center Rd
Canton, MI 48188
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SIGNED BY	SIGNED ON
Arqam Zia MD	12/02/2025

Referral electronically submitted by Arqam Zia MD 12/30/2025 01:04AM

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